



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان شوقى محمود على
المحافظة : القاهرة
الحى / المدينة : التبين
رقم البطاقة : 29303060103366 رقم التليفون : 01140374066
العنوان : ممممممممممممممممممممممم
العمر (تاريخ الميلاد) : 06-03-1993 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : اورام ليمفاويه
نتائج الأبحاث : (مرفق)

التشخيص النهائي : اورام ليمفاويه

خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي ونقل دم ومشتقاته
[13491]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: حنان شوقي محمود علي National ID: 29303060103366	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of lymphoma for Supportive and palliative Treatment and blood transfusion/blood products
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



حنان
شوقي محمود عني
١ الشارع الجديد، القبين البلد
القبين - القاهرة

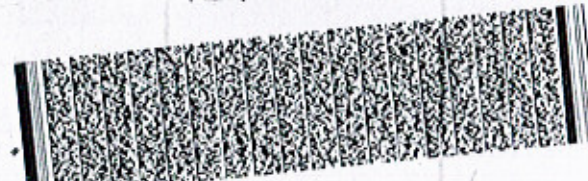
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رسم منزل



أنش
مسلمة
متزوجة
اسلام عبدالفتاح نادى عبدالواحد
البطاقة سارية حتى ٢٠٢٢/١١/٢٣

Patient Name: Hanan Shawky Mahmoud Ali	Date : 30/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 32 Year(s)
Medical ID: 2113/2020	Pat. ID: 16978

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS
REVEALED:

Known case of metastatic nasopharyngeal carcinoma

Compared to the last CT examination dated 28/7/2025 (hard copy with no DICOM series), the current study revealed:

I- CHEST:

- Bilateral apical fine reticulations denoting old granuloma are noted.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Regressive course regarding size of para-aortic lymph nodes seen encasing the aorta measures about 2.7 cm in its maximum thickness and 4.5 cm in CC dimension compared to 7cm.
- Regressive course of left para-aortic lymph node measures about 1.7x1.45 cm compared to 3x2.4 cm
- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.
- Still noted D8, D11 and D12 vertebral bodies sclerotic lesions likely metastasis.

Opinion:

- Regressive course regarding the size of previously noted abdominal lymph nodes.
- Stationary course of the rest of the examination.

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

Signature Dr.Mohamed Hassan
02/11/2025 3:46 AM



and Ali

Patient's Name:

Hanan

Date:

2026-08-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- Neck:

- Known Case of Nasopharyngeal Carcinoma seen as Focal heterogeneous enhancing lesion of mucosal thickening of the left nasopharynx with small central calcific dot .measures about 21 x13mm seen encroaching upon nasopharyngeal column with Obliteration, effacement of fossa of rosenmuller.
- Normal CT appearance of the parotid and submandibular salivary glands.
- Normal appearance of the thyroid gland.
- Normal appearance of the carotid sheath and its contents.

II – CHEST :

- Clear both lung fields with No visualized nodules or consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Noted dorsal spine vertebral spondylosis.

III- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal yet rather with low attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average size, and showing a smooth, regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, aorta, and IVC.
- Noted right ovarian cyst measuring (4.2x3.2cm) and IUD is seen in place inside uterine cavity yet Normal CT appearance of rest pelvic organs with no pelvic masses or collections
- The urinary bladder shows no evidence of gross masses, diverticulae, or stones.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- Known Case of Nasopharyngeal Carcinoma with lesion as described.
- Fatty Hepatomegaly
- Right ovarian cyst for Ultrasound correlation.
- Abdomen & Pelvis study showing No suspicious lesions.
- Clear both lung fields with No suspicious lesions.

GUSTAVE / ROUSSY

INTERNATIONAL

EGYPT

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MULTISLICE CT EXAMINATION OF THE CHEST (WITH CONTRAST)

- Lungs: Clear, with no evidence of pulmonary nodules, masses, consolidation, collapse, or cavitary lesions.
- Mediastinum & Hila: No hilar or mediastinal masses or lymph node enlargement.
- Pleura & Pericardium: No pleural or pericardial effusion.
- Cardiovascular System: Normal contrast-enhanced appearance of the heart and great vessels.

MULTISLICE POST-CONTRAST CT EXAMINATION OF THE ABDOMEN AND PELVIS

Reported case as metastatic NP carcinoma ; no previous imaging study for comparison

- Multiple pathologically enlarged upper abdominal lymph nodes are noted ,
 - largest seen wrapping the aorta at the level of D1 and DV12 ; averaging about 25 mm thickness and 7 cm in maximum CC length; seen partially encroaching upon the left exiting foramen , associated with related altered bony texture of adjacent vertebrae.
 - left supra renal para aortic lymph node reaching 30x 24 mm .
- Liver: Average in size with homogeneous parenchymal attenuation. No focal lesions or biliary ductal dilation.
- Kidneys: Normal in size, position, and morphology with smooth cortical outlines and preserved parenchymal thickness. Nephrogram and excretory function are unremarkable. No evidence of calculi or hydronephrosis.
- Spleen, Pancreas, and Adrenal Glands: Appear normal with no focal abnormalities.
- Major Vessels (Aorta & IVC): Normal caliber with no evidence of thrombosis or aneurysm.
- Urinary Bladder: Well-distended, with no evidence of masses, diverticula, or calculi.
- Pelvic Organs IUD is noted Normal appearance with no detected masses or fluid collections.
- Ascites: minimal pelvic collection

Scan through the examined bone revealed :

- DV7 , DV11 and DV12 altered vertebral with sclerotic and lytic osseous lesions .. metastasis .

Dr. Amal Abouhelal, MD

"Baseline before Gen (cis)"

GUSTAVE / ROUSSY /

INTERNATIONAL
EGYPT

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